

D17 — Cardiology Reconsult

Dr. A. Kim, Cardiology | Cardiology | 02/25/2026 10:00

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

CARDIOLOGY RECONSULT NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/25/2026 10:00 | Provider: Dr. A. Kim, Cardiology
Reason for Reconsult: CHF medication optimization post-GI bleed / AKI

CURRENT STATUS: Cr 1.9 (down from peak 2.3). K+ 4.6. Hgb 8.5 (stable).
Hemodynamically stable.

RECOMMENDATIONS:

1. Carvedilol: Continue 12.5 mg BID (reduced from 25 mg during ICU for hypotension). Uptitrate as outpatient when hemodynamically stable.
2. Lisinopril: Recommend restart at 10 mg daily (reduced from home dose 20 mg). Cr trending down and has been stable x 48 hours. CHF mortality benefit of ACEi is substantial in HFrEF. Risk of not restarting (CHF decompensation, progressive remodeling) outweighs risk of modest GFR decline.
3. Spironolactone: Recommend restart at 25 mg daily. K+ 4.6 today – acceptable to restart. Mortality benefit in HFrEF is well-established. Monitor K+ closely (Q48H initially).
4. Furosemide: Restart at 20 mg PO daily. Patient is currently euvolemic (no longer volume overloaded, no longer hypovolemic). This is a reduced dose from home (40 mg). Reassess volume status and dose as outpatient.
5. Apixaban: Defer to IR for restart timing given embolization procedure. Once cleared, restart at home dose 5 mg BID. CHA2DS2-VASc score of 5 (age 65-74, CHF, HTN, DM, vascular disease) – stroke risk is substantial and outweighs re-bleeding risk once hemostasis is confirmed durable.
6. Follow up with heart failure clinic within 2 weeks of discharge for GDMT optimization.

Electronically signed by: Dr. A. Kim, MD
Date: 02/25/2026 10:30

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